

Market Dynamics, Regulatory Shifts, and the AI Paradigm

Overview

The **health insurance industry** is the sector made up of companies that **sell health insurance plans** to **protect people from high medical costs** by sharing the financial risk. Health Insurance Industry in India is one of the **fastest growing segment** with increasing penetration rates and diverse policy offerings



market size in terms of Gross Written Premium (GWP)
~USD 15.06 billion in 2024



Compound Annual Growth Rate (CAGR) of
20.9% from 2025 to 2030



India's total insurance penetration
4% in FY23
(global average of 6.8%)



India's Population Coverage Around
51%
ranks around **25th** in the world

Penetration Levels



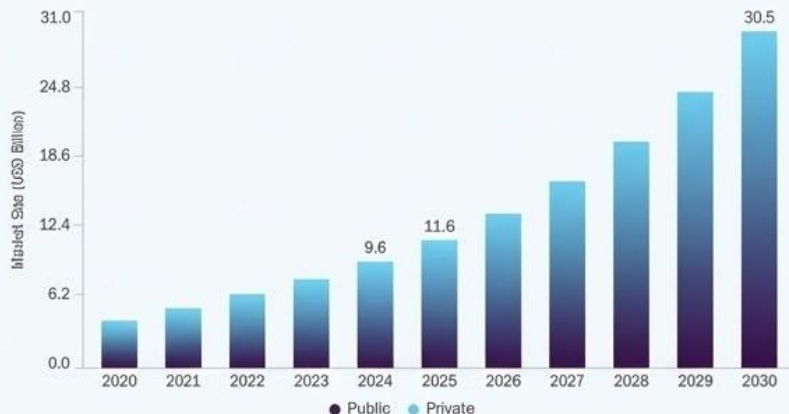
Urban Area
62%



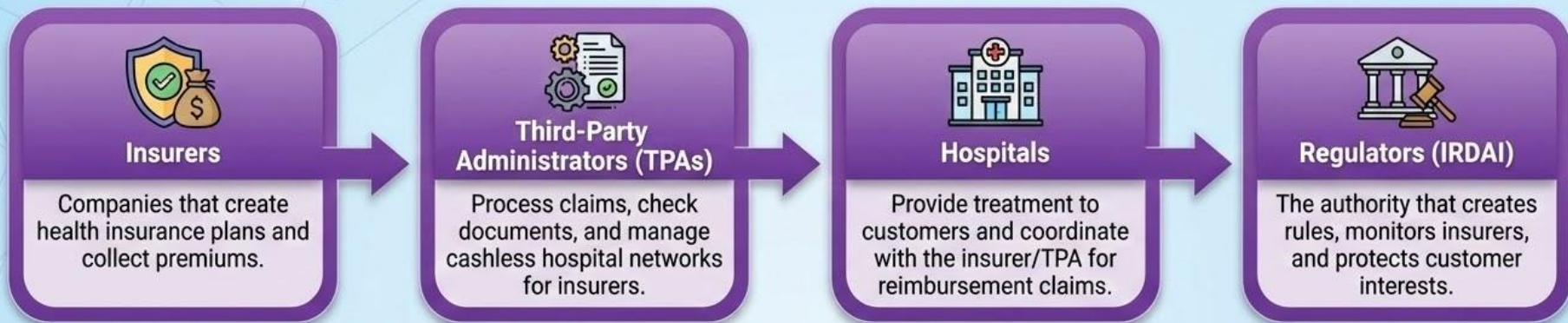
Rural Area
41%

India Health Insurance Market

Size (NBP), by Insurance Type, 2020 - 2030 (USD Billion)



Market Structure & Value Chain



Players



Public Sector Undertakings (PSUs):

Government-owned insurers. They are large, trusted, but grow slowly and operate with older systems.



Standalone Health Insurers (SAHIs):

These are companies focused only on health insurance, they offer deeper health expertise, better underwriting, and strong retail penetration.



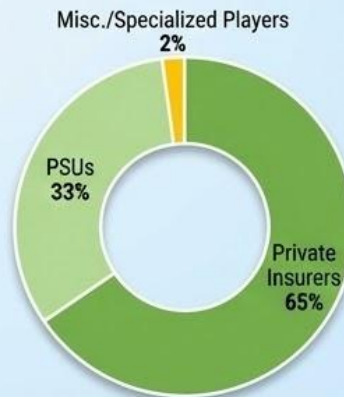
Private Multi-line Insurers:

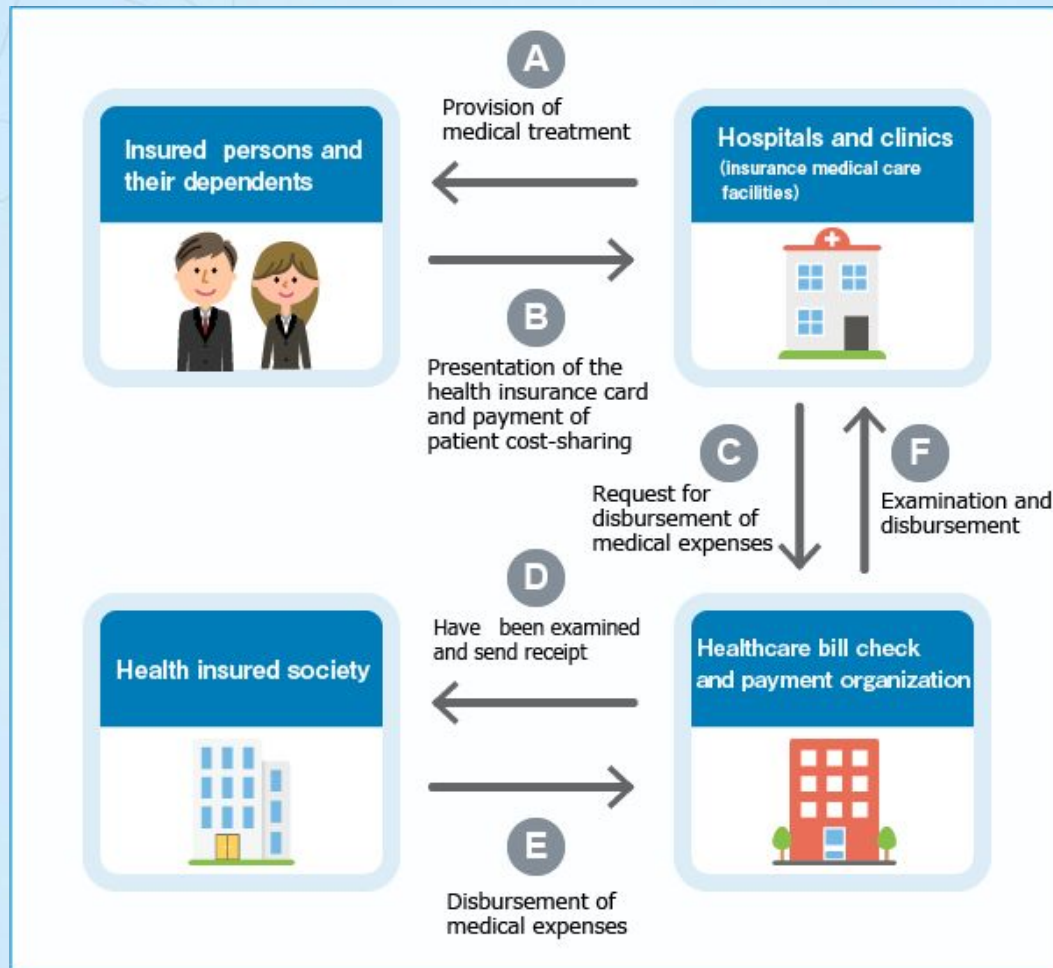
Privately owned companies that sell all kinds of general insurance including health. They grow faster due to better technology, wider product variety, and aggressive marketing.



Insurtechs:

Tech-driven players, They disrupt the market through digital distribution, seamless onboarding, subscription-based health plans, and improved customer experience.





Regulatory Landscape

IRDAI (Insurance Regulatory and Development Authority of India) is the national regulator for insurance.

Key Mandates



(1) Pricing & Product Regulation

Standardization of terms, limits on premium changes, approval needed for new products.



(2) Solvency Requirement

Insurers must maintain 150% solvency ratio to ensure financial stability.



(3) Consumer Protection

Timelines for claim settlement, grievance redressal norms, & obligations for transparency in policy wording.



(4) Health Guidelines 2020

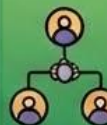
Clear rules on pre-existing diseases, claim settlement timelines, standardized coverage definitions.

Reforms



(1) Regulatory Sandbox

Allows insurers to test innovative products/services, pricing models, distribution methods, or technology for a limited period under relaxed regulations.



(2) Bima Trinity



Bima Vahak

Insurers committed to provide airtight records and transparent changes.



Bima Sugam

Insurers must amount of interest services, in health services, correct payments.



Bima Vistaar

Insurers must claim report connection and economics to health insurers.



(3) Digital Health Stack (ABDM)

Digitization of health records, e-KYC, claim exchange to improve interoperability.

Risk Pooling & Pricing Concepts

What is Risk Pooling?



Many people pay small premiums into a common fund (the risk pool). When a few members need care, their bills are paid from that pool – spreading the financial burden across many people.

Key Market Failures



Adverse Selection

People who expect high medical costs are more likely to buy insurance, which raises average claim costs and pushes premiums up.
(Source: IDEAS/RePEc)



Moral Hazard

Once insured, people may consume more healthcare (more tests, admissions) because they don't bear the full cost, increasing overall payouts.
(Source: IDEAS/RePEc)

How Insurers Manage These Problems



Underwriting & medical checks
(to exclude/price pre-existing conditions)



Waiting periods & exclusions for pre-existing diseases



Co-payments / deductibles to reduce moral hazard



Group / employer schemes to diversify risk



Use of digital health data / ABDM for better risk scoring

Why large & diverse pools matter



Large, Diverse Pool (Stable)

Large, mixed pools (young + old, healthy + sick) stabilize average claims and keep premiums predictable.



Predictable Premiums



Small or Skewed Pool (Volatile)

Small or skewed pools make premiums volatile.



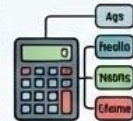
Volatile Premiums

Pricing Approaches (Contrast)



Community Rating

Everyone in the community pays the same or similar premium (simple, equitable but can encourage adverse selection).



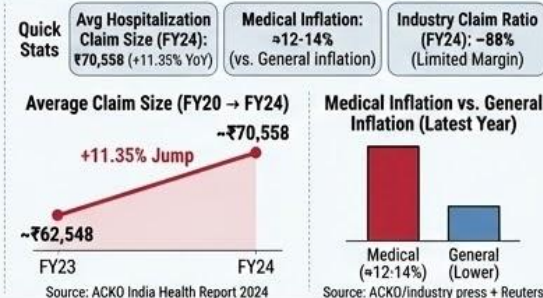
Risk-based Pricing

Premiums vary by age, health, habits (smoking, BMI) and past claims – more actuarially fair but needs good data and regulator comfort.

Evidence & Stats

Short Empirical Evidence

- A study of Indian insurance markets finds both adverse selection and some moral hazard effects – insured people have higher hospitalization rates...
(Source: IDEAS/RePEc)
- Large field experiments on government schemes (RSBY) show the extent of selection depends strongly on product pricing...
(Source: J-PAL)



Balanced, large risk pools + better data (ABDM) + careful pricing are essential to avoid adverse selection, control moral hazard, and keep premiums sustainable.

Key Cost Drivers

Medical Inflation



- Medical inflation in India is consistently high ~12–14% per annum for hospital costs, diagnostics, medicines, and care delivery.
- The trend forces insurers to periodically raise premiums to keep up with rising treatment and hospitalization costs.

Claim Costs: Hospitalization, Chronic Diseases



- The average health-insurance claim size in India rose to ~**₹70,558** in FY 2023-24, up from ~**₹62,548** in FY 2022-23 – that's an 11.35% jump in one year.
- Serious diseases lead to high-cost claims – which increases the average payout by insurers, and in turn pressure premiums upward.

Underwriting Risk: Ageing Population, Comorbidities



- As the population ages and chronic diseases rise (like heart disease, diabetes, kidney disorders, and cancer), high-cost claims become more frequent, raising overall risk for insurers.
- More high-risk customers (older, comorbid) in the insured pool means insurers must price accordingly to avoid losses, which pushes premiums higher.

Fraud & Leakage / Overuse of Services



- Rising treatment costs and practices like fraud, over-billing, unnecessary procedures, and avoidable admissions increase overall claim payouts—raising costs for everyone, even those who don't file claims.
- Losses of about **₹8,000-10,000 crore** annually due to fraud; 3–4% of all claims are identified as fraudulent.

Consumer Behavior & Demand Trends (Post-COVID)



1. Rising Risk Awareness

COVID heightened awareness of medical uncertainty → more people buying health cover and upgrading sums insured.



2. Shift to Family-Floater Plans

Families prefer single, shared coverage instead of multiple individual plans → cost-effective + wider protection.



3. Strong Demand for Cashless Treatment

Consumers prioritize insurers with large hospital networks and seamless cashless claim approvals to avoid out-of-pocket payments.



4. Digital-First Buying Behavior

Policy purchases shifting online: comparison via aggregators, instant issuance, e-KYC, and digital servicing.

5. Tier-2 & Tier-3 Market Growth



Faster adoption driven by rising awareness, pandemic experience, and more affordable premium products.

6. Preference for OPD + Preventive + Wellness Benefits



People want cover beyond hospitalization – OPD consultations, diagnostics, telemedicine, wellness rewards.

7. High Price Sensitivity

Premium affordability remains critical; EMI options and budget plans (Arogya Sanjeevani, OPD riders) gaining traction.



30–35%

of policies FY22–24 bought online (industry estimates).



> 60%

Cashless claims now of all claims in retail health.



~55%

Tier-2/3 cities account for of new retail health policies (IRDAI/industry reports).



₹5–10 lakh

Rising preference for cover vs earlier ₹2–3 lakh.

Challenges



1. Rising medical inflation & high claim ratios

- India has one of the highest medical inflation rates (14–18%) globally. Proin in tellus felis volutpat



3. Limited Data Standardization Across Hospitals

- Hospitals maintain fragmented, non-standardized medical records. Checking claims, understanding medical history, and calculating premium risk becomes complicated and slow.



5. Low awareness & mis-selling

- Many people still rely on out-of-pocket spending. Low understanding of terms like deductibles, co-pays, pre-existing disease waiting periods → dissatisfaction and mistrust.



7. Difficulty in Reaching Tier-2 & Tier-3 Cities

- Limited network hospitals, Lower awareness. Smaller disposable incomes, Rural markets remain underserved.



2. Fraudulent claims & leakages

- Fake bills, inflated costs, unnecessary admissions, overcharging by hospitals. Fraud estimated to contribute 10–15% of claims cost.



4. Price wars leading to thin margins

- Aggressive competition among insurers → underpricing premiums to acquire customers. Thin margins leave no room for innovation or buffers for medical inflation.



6. High Dependency on Agents & Intermediaries

- Traditional distribution relies heavily on agents → higher commission costs. Mis-selling by intermediaries leads to customer distrust and higher complaint.



8. Long claim settlement cycles

- Long waiting times during hospitalization authorizations. Delayed reimbursements, document-heavy processes. Cashless claim approvals inconsistent across hospitals.

Opportunity Areas / Strategic Levers



1. AI-Driven Underwriting

Use machine learning on medical history, wearable data, and claim patterns to assess risk more accurately → faster onboarding + reduced underwriting costs.

2. Personalized & Dynamic Pricing

Leverage digital health records (ABDM), lifestyle indicators, and past utilization to offer fair, customized premiums instead of broad community pricing.



STRATEGIC LEVERS & OPPORTUNITIES



4. Preventive Health Ecosystem Partnerships

Collaborate with gyms, diagnostic chains, nutrition platforms, physiotherapy centers to encourage healthier behavior and reduce long-term claim costs.



5. Wellness-Linked Rewards & Premium Discounts

Offer incentives for steps tracked, preventive tests, compliance with treatment, and chronic disease management → lowers risk and improves retention.



3. Embedded Health Insurance

Seamless insurance bundled with fintech apps, travel bookings, ecommerce purchases, telemedicine platforms → expands reach and reduces acquisition cost.

Thank You

